SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Pertuzumab + trastuzumab + vinorelbine

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

HER2-positive advanced breast cancer · taxanes unsuitable · NCCP 00526 · version 5 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Control the cancer and symptoms, with repeated review of benefit and tolerance.

Schedule
IV pertuzumab and IV trastuzumab on Day 1; vinorelbine on Days 1 and 8 of each 21-day cycle. Vinorelbine is prescribed by IV OR by mouth, not both. There is no planned Day 15 vinorelbine. Continue while beneficial and tolerated; treatment may change if one component causes toxicity.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Duration and treatment setting discussed
- ☐ Supportive medicines and urgent contact advice discussed
- ☐ Common, serious and lasting agent-specific risks discussed below
- ☐ Correct HER2 medicine and formulation confirmed
- ☐ Heart and other required monitoring explained
- ☐ Pregnancy / contraception / breastfeeding discussed where relevant
- ☐ Companion cancer medicines reviewed using their own information

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pertuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction
- ☐ Treatment reaction
- ☐ Diarrhoea and rash
- ☐ Fatigue

Trastuzumab
HER2-TARGETED ANTIBODY

- ☐ Heart pumping-function reduction
- ☐ Treatment reaction
- ☐ Diarrhoea and rash
- ☐ Fatigue

Vinorelbine
VINCA-ALKALOID CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Constipation or bowel obstruction
- ☐ Numbness or weakness
- ☐ Nausea; oral doses need a specific plan
- ☐ Irritation if an IV dose leaks

Discuss the treatment aim, alternatives, expected benefit, common and serious effects, fertility and pregnancy precautions, route, monitoring, practical support and the agreed contact plan. The treating team completes the approved individual consent process.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00526 v5.

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